

EROSIVE GINGIVOSTOMATITIS (DESQUAMATIVE GINGIVITIS)

Erosive gingivostomatitis is characterized by inflammation and erythema of the gingiva and represents a nonspecific reaction pattern. It may be associated with viral infections or autoimmune, inflammatory, and blistering disorders such as lichen planus (LP), erythema multiforme, pemphigus, and cicatricial pemphigoid. Gingivitis and periodontitis are specifically excluded from this diagnosis. Other mucocutaneous sites may also be involved.

LICHENOID MUCOSITIS

Lichenoid mucositis presents with reticulated white plaques and painful erythematous erosions on mucosal surfaces. It can result from lichen planus, medications (e.g., nonsteroidal anti-inflammatory drugs, antihypertensives), contact allergy, or graft-versus-host disease. Identification and management of underlying causes or triggers are essential. If symptoms persist after removing the etiology, treatment may progress from topical steroids to systemic corticosteroids.

LICHEN PLANUS

Lichen planus (LP) is a common, chronic, inflammatory condition affecting the skin and mucosa, predominantly in adult women. It is characterized by lacy, white patches, plaques, or papules, which may be asymptomatic or painful, and often accompanied by erosions and ulcerations.

ACUTE NECROTIZING ULCERATIVE GINGIVITIS (TRENCH MOUTH, VINCENT DISEASE)

Acute necrotizing ulcerative gingivostomatitis is marked by punched-out ulcers of the interdental papillae. Precipitating factors include stress, poor oral hygiene, nutritional deficiencies, and alcohol or tobacco use. Immunosuppressed

individuals, especially those with HIV, are at higher risk. Common features include gingival hemorrhage, severe pain, foul odor, fever, and lymphadenopathy. In advanced cases, it may lead to alveolar bone destruction. Associated microorganisms include *Treponema*, *Selenomonas*, *Bacteroides fusiformis*, *Prevotella intermedia*, and *Borrelia vincentii*. Treatment involves improved oral hygiene, antiseptic rinses, systemic antibiotics, and, in severe cases, surgical debridement.

GINGIVAL HYPERPLASIA

Gingival hyperplasia refers to enlargement of the free and attached gingiva, particularly the interdental papillae. It may result from infiltration by malignant lymphocytes (e.g., in leukemia), glycogen storage disease type Ib, or medications such as phenytoin, calcium channel blockers, and cyclosporine. Management includes discontinuation of the causative agent if possible, improved oral hygiene, antibiotics, and gingivectomy in moderate to severe cases.

APHTHOUS ULCERATION (APTHAE, CANKER SORE, RECURRENT APHTHOUS ULCERATION)

Aphthous ulceration is characterized by acute, recurrent, painful ulcers occurring on nonkeratinized oral mucosa. It is the most common cause of oral ulcers, affecting up to 30% of the population. Most individuals experience episodic ulcer development.